

Histology: Carcinoma, Urothelial, NOS

8/20/3

lw

11/22/10

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TCGA-BL-A13J-01A-PR

Redacted



Path Site: bladder, NOS C67.9

CQCF: bladder wall, posterior C67.4

SPECIMEN

- A. Right distal ureter
- B. Left distal ureter
- C. Left pelvic lymph node
- D. Bladder and prostate
- E. Right pelvic nodes
- F. Left pelvic nodes
- G. Appendix
- H. Sigmoid colon

CLINICAL NOTES

CLINICAL HISTORY: Bladder cancer.

PRE-OP DIAGNOSIS: Bladder cancer.

FROZEN SECTION DIAGNOSIS

- A) Ureter, right distal, excision - Inflammation with variable epithelial atypia.
- B) Ureter, left distal, excision - Inflammation with variable epithelial atypia.
- C) Lymph node, left pelvic, excision - Metastatic carcinoma.

GROSS DESCRIPTION

A. Received fresh for frozen section labelled "right distal ureter" is a segment of ureter that measures 0.4 x 0.4 x 0.3 cm in dimension. Tissue is entirely frozen.

B. Received fresh for frozen section labelled "left distal ureter" is a segment of ureter that has a dilated lumen and measures 0.8 x 0.7 x 0.3 cm in dimension. Tissue is entirely frozen.

C. Received fresh labelled "left pelvic lymph node" is an irregular-shaped fragment of fibroadipose tissue that measures 3.5 x 1.5 x 1.2 cm in dimension. Within the tissue, a lymph node is identified that measures 1.7 cm in dimension. The lymph node is

GROSS DESCRIPTION

entirely frozen as FSC1 and FSC2.

D. Received fresh and subsequently fixed in formalin labeled "bladder and prostate" is a 21 x 10 x 6.5 cm bladder and prostate, which is partially covered with abundant yellow lobular fat. The bladder itself is approximately 8.5 x 7.8 x 6 cm and the prostate is approximately 35 grams, 4.5 x 3.5 x 3.2 cm.

The left ureter is approximately 5 cm x 0.7 cm and is markedly dilated. The right ureter is approximately 4.5 x 0.5 cm. There are adhesions on the posterior aspect of the bladder wall in a 4 x 3.5

cm area. The right half of the specimen is inked blue and the left posterior aspect of the specimen is inked black. The left anterior aspect of the specimen is inked also blue. The specimen is opened in a Y-shaped incision to show a friable area of tumor in the posterior wall. There is also tumor present in the left lateral wall. This entire area is 6.7 x 6.5 cm. This comes within 5.5 cm of the prostatic urethral margin. There is also a tumor polyp measuring 1 cm in the right anterior part of the wall, which is located 7 cm from the prostatic urethra. The cut surface of the polyp showed no discrete invasion through the wall; however, the cut

surface of the large tumor mass shows invasion through the left lateral wall and through the posterior wall. This comes within 0.1 cm of the radial margin. No discrete tumor is grossly identified within the left ureter; however, the ureter is markedly dilated up to 0.7 cm in diameter. No lymph nodes are grossly identified in

the surrounding fat. The seminal vesicles average 3.5 x 1.5 x 0.8 cm. The vasa deferentia could not be grossly identified. Representative sections of the specimen are submitted following further fixation.

BLOCK SUMMARY: 1 - apical bladder margin with prostatic urethral margin en face; 2-3 - prostatic urethra, with full-thickness prostate composite slice; 4 - bilateral ureteral margins and representative seminal vesicles; 5 - representative anterior bladder; 6 - representative dome of bladder to tumor,

GROSS DESCRIPTION

full-thickness; 7 - trigone full-thickness; 8 - representative section of bladder to posterior margin full-thickness; 9 - right side bladder to ureteral orifice; 10 - right posterior bladder full-thickness; 11-12 - left side full-thickness composite bladder with ureteral orifice; 13 - possible representative left side vas deferens within tumor and representative section of right-side seminal vesicle; 14 - additional representative section of full-thickness tumor to posterior wall; 15 - entire right-sided polyp full-thickness; 16-22 - representative section of prostate as follows: Block 1 - full-thickness composite; 17-18 - full-thickness composite; 19-20 - full-thickness composite; 21-22 - full-thickness composite. RS-22.

E. Received fresh, subsequently fixed in formalin labeled "right pelvic nodes" is a 4.5 x 3.2 x 2.0 cm aggregate of yellow lobular fatty tissue. Lymph nodes are grossly identified which range from 0.5 cm to 3.5 cm in greatest dimension. The lymph nodes are entirely submitted as follows:

BLOCK SUMMARY: 1 - three possible lymph nodes; 2 - one possible lymph node bisected; 3-4 - one possible lymph node bisected, one-half in each.

F. Received fresh, subsequently fixed in formalin labeled "left pelvic lymph nodes" is 4.5 x 4.0 x 2.2. The specimen is palpated to identify multiple lymph nodes which range from 1.2 cm to 7.5 cm in greatest dimension. The lymph nodes are entirely submitted as follows:
BLOCK SUMMARY: 1 - three possible lymph nodes; 2-5 - one possible lymph node sectioned.

G. Received fresh, subsequently fixed in formalin labeled "appendix" is a 5.5 x 0.5 cm appendix which is partially covered with pink-tan smooth glistening serosa and yellow lobular fat. The specimen is sectioned to show an intact wall with an average thickness of 0.3 cm. The lumen ranges from pinpoint to 0.2 cm. No other discrete gross lesions are identified. The appendix is

GROSS DESCRIPTION

entirely submitted in two cassettes with the proximal end inked (fat and staples retained). AS-2.

H. Received fresh subsequently fixed in formalin labeled "sigmoid colon" is a 14.5 cm. long portion of colon which is opened at one end inked blue and the opposite end is stapled and is inked black. The serosa of the specimen is pink tan, smooth and glistening and is partially covered with abundant yellow lobular fat. There are a few adhesions on the specimen, however, no hemorrhage or fibrinous exudate is grossly identified. The specimen is opened to show pink tan smooth glistening mucosa with an average circumference of 4.5 cm. There is a 0.3 cm. sessile polyp which is located on a fold, 4.5 cm. to the blue inked margin. A few diverticula are grossly identified and show no gross evidence of submucosal abscesses or perforation present. Representative sections of the specimen are submitted as follows:

BLOCK SUMMARY: 1 - representative luminal margins; 2 - sessile polyp; 3 - representative normal; 4 through 8 - representative sections of the remainder of the specimen including diverticula.
RS-8

MICROSCOPIC DESCRIPTION

Histologic type: Urothelial carcinoma.
Histologic grade: High grade
Associated epithelial lesions: Papillary urothelial carcinoma, high

grade and carcinoma in situ.
Primary tumor (pT): Tumor invades through the full thickness of the wall into the adjacent adipose tissue and invades into prostatic stroma from the prostatic urethra, pT4.
Margins of resection: Tumor extends to the soft tissue margin of excision on the posterior surface of the bladder and is present at

MICROSCOPIC DESCRIPTION

the prostatic urethral margin.
Regional lymph nodes (pN): 2 of 11 lymph nodes positive for metastatic carcinoma (2/11), pN2.
Distant metastasis (pM): pMx
Vascular invasion: Present.
Other findings: Perineural invasion by tumor is present. Tumor is present surrounding the left side vas deferens. The prostatic urethra shows changes of carcinoma in situ and invasive urothelial carcinoma extending from glands into prostatic stroma. The prostate shows glandular and stromal hyperplasia.

14x3, 15x1, 3x3, 5x1, 4x3, 2x1

DIAGNOSIS

- A. Urethra, right distal, excision - Focal urothelial atypia with associated inflammation.
- B. Urethra, left distal, excision - Focal urothelial atypia with associated inflammation.
- C. Lymph node, left pelvic, excision - 1 lymph node positive for metastatic urothelial carcinoma (1/1).
 - The size of the metastasis is 1.7 cm. in greatest dimension.
 - Extracapsular extension by tumor is identified.
- D. Bladder and prostate, excision - Invasive poorly differentiated urothelial carcinoma involving the bladder and prostatic urethra.
 - Tumor extends through the full thickness of the wall into the adjacent soft tissue and is present at the posterior soft tissue margin of excision.
 - Tumor extends to the prostatic urethral margin of excision.
 - Tumor is present within the prostatic urethra and invasive urothelial carcinoma is present in prostatic stroma (see tumor characteristics in the prostatectomy template).
- E. Lymph nodes, right pelvic, excision - 1 of 4 lymph nodes positive

DIAGNOSIS

for metastatic carcinoma (1/4).

F. Lymph nodes, left pelvic, excision - 6 lymph nodes negative for metastatic carcinoma (0/6).

G. Appendix, appendectomy - No significant histopathologic abnormality.

H. Colon, sigmoid, excision - Diverticulosis with diverticulitis with focal changes consistent with chronic diverticulitis with associated fibrosis in the pericolic adipose tissue with inflammation and reactive changes.

Adenomatous polyp.

The margins of excision do not show adenomatous change.

(Electronic Signature)

--- End Of Report ---

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
IPSA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case Is (Circle):		
Reviewer Initials		
Date Reviewed:		

QUALIFIED